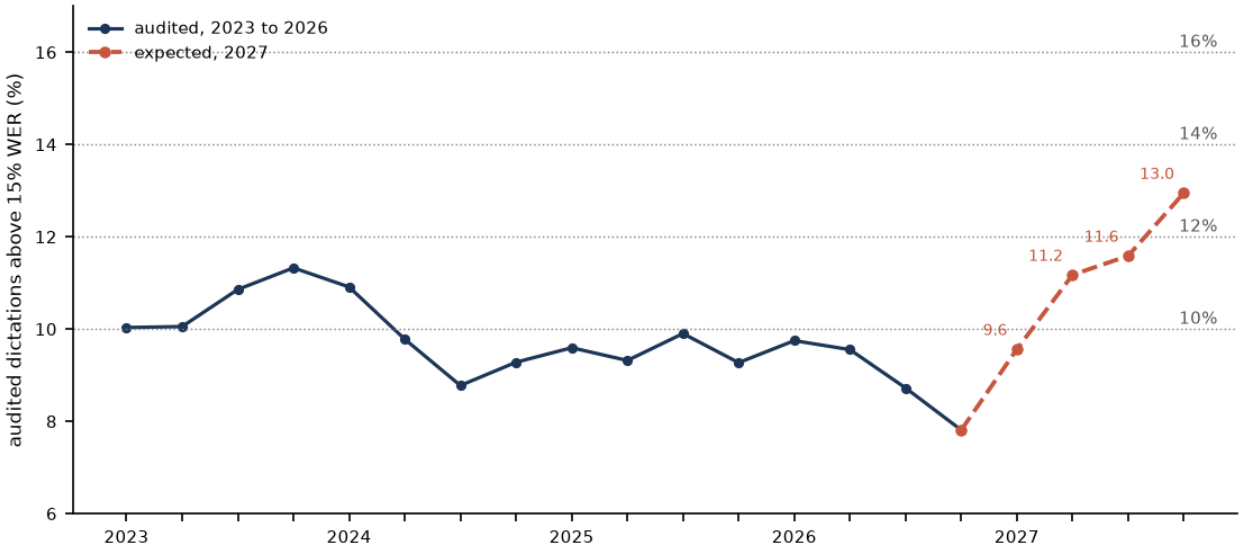
Shannonside 2027 quality schedule: the heavy-correction ceiling

12 January 2027. Jody Cahir, Head of Recognition Quality, for the account team.

We should commit to Ceiling C, 14 per cent.

A 10 per cent ceiling would be passed in the second quarter of 2027 and a 12 per cent ceiling in the fourth. No quarter is expected to pass 14 per cent: the fourth, the highest, comes to about 13.0 per cent.



2027	Expected share above 15%	Lowest ceiling it stays under
Q1	9.6%	10%
Q2	11.2%	12%
Q3	11.6%	12%
Q4	13.0%	14%

Audited shares to the end of 2026 from the QA-07 audits; 2027 expected from the audits, the group's HR register and the six past go-lives, with recognition on release 6.0 through 2027 (SN-2026-14).

Why the record does not carry forward

The audited share went from 10.1 per cent in the second quarter of 2023 to 7.8 per cent at the end of 2026. Measured within clinicians, releases 5.0, 5.2 and 6.0 together cut a clinician's error by about 10.6 per cent, which on its own would have taken about 4.1 points off the share; the changing mix of dictations put about 1.9 back, as the six smaller hospitals went compulsory and the clinicians who had not chosen to dictate joined. 2027 has no release (SN-2026-14) and brings the two university hospitals into the service: Shannonside University from 1 February 2027 and Ballykeel University from 1 October 2027. Most of their staff do not dictate yet.

The university hospitals

	Shannonside University	Ballykeel University
In post, 31 December 2026	818	1,250
Dictating in late 2026	187	332
Starting once compulsory	631	918
The hospital's own effect on error	+5.0%	+8.6%
Heavy-correction share, 2027 starters	18.2%	21.7%
Expected share, 2027 Q4	14.9%	12.9%

In post: the HR register at 31 December 2026, as the rollout plan counts them. Dictating: any dictation from October to December 2026.

The clinicians dictating there now look excellent (4.1 and 4.4 per cent heavy correction) because they are the few who chose to. The 248 moves between the group's hospitals that the register records under a new staff number let us separate the place from the people: the same clinician's word error rate is about 5.0 per cent higher at Shannonside University and 8.6 per cent higher at Ballykeel than at the six smaller hospitals on average. The starters are most of each staff, not the hard core who held out at the smaller hospitals (whose compelled clinicians carry 27.7 per cent heavy correction): their share is the whole staff's, at the smaller

hospitals' levels for their grades and specialties with the hospital's own effect, less what the volunteers already carry.

The go-lives switch over department by department. At the six earlier go-lives the compelled clinicians dictated about 15, 36 and 64 per cent of a settled month in the first three months, so Ballykeel's fourth quarter holds about 88,000 dictations, 43,000 of them from its starters.

Nine in ten of the fourth quarter's dictations are expected to stay under 16.1 per cent word error rate.

What we ruled out

Carrying 2026's 9.0 per cent forward points to 10 per cent. Putting the university hospitals' starters at the smaller hospitals' compelled clinicians' rate puts the fourth quarter at 16.1 per cent, above every ceiling offered. Reading the university hospitals at the smaller hospitals' acoustic level gives 11.4 per cent and 12. Starting the go-lives at full volume gives 14.6 per cent and 16.

Heavy correction is an audited dictation whose word error rate is above 15 per cent, the adjudicated score where a dictation was scored twice (draft Schedule 4.1; QA-07 4). Quarters are calendar quarters by dictation date.